MEDINOVA TEACHING HOSPITAL

Class 18 — Dermatology & Skin Emergencies

Skin Emergencies · Wound Assessment · Drug Reactions · Common Dermatoses

> *"The skin is the window through which every systemic disease eventually appears."*

BOARD CERTIFICATION SERIES | MODULE 18 of 20

SECTION 01

Dermatological Terminology

Precise language is the key to remote consultation and professional documentation.

Primary Lesion	Definition	Clinical Example
Macule	Flat, < 1cm. Color change only.	Freckles, Vitiligo.
Papule	Raised, solid, < 1cm.	Acne, Plane warts.
Plaque	Raised, flat-topped, > 1cm.	Psoriasis.
Vesicle	Fluid-filled, < 1cm.	Herpes Simplex, Varicella.
Bulla	Fluid-filled, > 1cm.	Bullous Pemphigoid, Burns.
Wheal	Transient, oedematous papule/plaque.	Urticaria (Hives).

> **Prof. Obi's Pearl #1** "If you can't describe the lesion, you can't diagnose the disease. Is it a vesicle or a pustule? Fluid or pus? That one distinction changes your entire management."

SECTION 02

Life-Threatening Skin Emergencies

1. SJS (< 10% BSA) vs TEN (> 30% BSA):
Epidermal necrolysis. **Nikolsky sign (+ve).**
SCORTEN: Age > 40, HR > 120, Malignancy, BSA > 10%, Urea > 10, Glucose > 14, Bicarb < 20. (Score ≥ 5 = 90% mortality).

2. Necrotising Fasciitis: "Pain out of proportion to skin findings." Wood-hard swelling.
LRINEC Score: CRP > 150, WBC > 15, Hb < 11, Na < 135, Glucose > 10, Creatinine > 141. (Score ≥ 6 = Suspect Nec-Fasc).

3. Meningococcal Purpura: Non-blanching purpuric rash + Sepsis. Requires immediate IM/IV Ceftriaxone before transport.

SECTION 03 & 04

Drug Reactions & Sub-Saharan Africa

DRESS Syndrome

Drug **R**eaction with **E**osinophilia and **S**ystemic **S**ymptoms. High fever, rash, and liver/renal failure 2-8 weeks post-drug.

Common Dermatoses in Nigeria

Condition	Features	Treatment
Pityriasis Versicolor	Hypo/Hyperpigmented scaly patches.	Ketoconazole shampoo/cream.
Scabies	Nocturnal itch, burrows in webs.	Permethrin 5% (Neck-down).
Keloid Scarring	Overgrowth beyond wound margin.	Intralesional steroids, Silcone.
PIH	Darkening after inflammation.	Sunscreen (critical), Azelaic acid.

> **Prof. Obi's Pearl #2** "In dark skin tones, erythema (redness) may appear purple or dark brown. You must look for texture and warmth, not just color, or you will miss early cellulitis."

SECTION 05 & 06

Wound Assessment & Skin Cancer

MEASURE Framework: **M**easure, **E**xudate, **A**ppearance, **S**uffering, **U**ndermining, **R**e-evaluate, **E**dge.

ABCDE of Melanoma

- **A – Asymmetry:** One half unlike the other.
- **B – Border:** Irregular, scalloped, or poorly defined.
- **C – Color:** Varied shades (brown, black, red, blue).
- **D – Diameter:** Usually > 6mm.
- **E – Evolving:** Changing in size, shape, or color.

> **Prof. Obi's Pearl #3** "Breslow thickness is the single most important prognostic factor in melanoma. Every millimeter matters. If you suspect it, excise it—don't just punch biopsy the middle."

SECTION 07

Systemic Diseases: Cutaneous Signs

Systemic Disease	Cutaneous Presentation
Diabetes Mellitus	Acanthosis Nigricans, Necrobiosis Lipoidica.
SLE	Malar (Butterfly) rash, Photosensitivity.
Sarcoidosis	Lupus Pernio, Erythema Nodosum.
Hyperthyroidism	Pre-tibial Myxoedema.
Vasculitis	Palpable Purpura.

> **Prof. Obi's Pearl #4** "Acanthosis nigricans isn't 'dirt' on the neck. It is the skin screaming that the patient's insulin resistance is out of control. Treat the metabolism, not the skin."

OFFICIAL SIGN-OFF

ISSUED BY: Lead Dermatologist, MediNova Cutaneous Unit
INSTITUTION: MediNova Teaching Hospital
SERIES: Board Certification Module 18 of 20
DATE: March 2026